



UHL Child Death Paperwork & CDOP Process (0-18 years)

**For use in ALL child deaths outside of
the Neonatal unit and Labour ward**

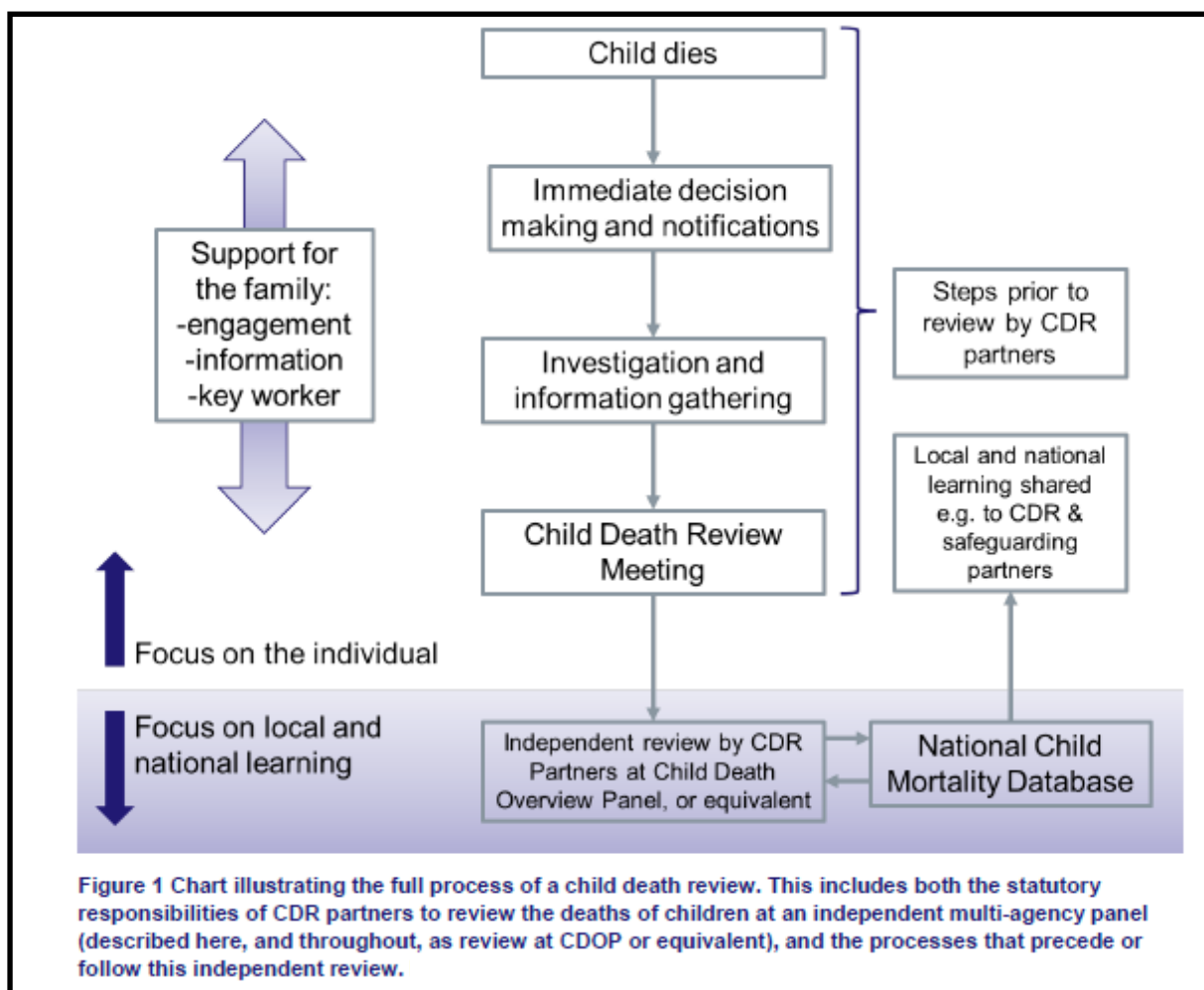


The aim of this booklet is to take teams in a step by step process through the procedures and policies required by law following the death of any child before their 18th birthday. It has been designed to be used on children's wards and also in the emergency department (it may also be used for under 18's who die on adult wards).

When completed it will provide necessary information for external partners, aid teams in providing feedback and support to families and provide a structured format for staff to follow in a stressful situation.

The [statutory guidance](#) (last updated in 2018) requires that all deaths of under 18's undergo a formal review.

After completion of this booklet it should be placed in the patients notes. It may be used later (along with the child's medical records) to assist in reviewing care and answering any questions the family may have.

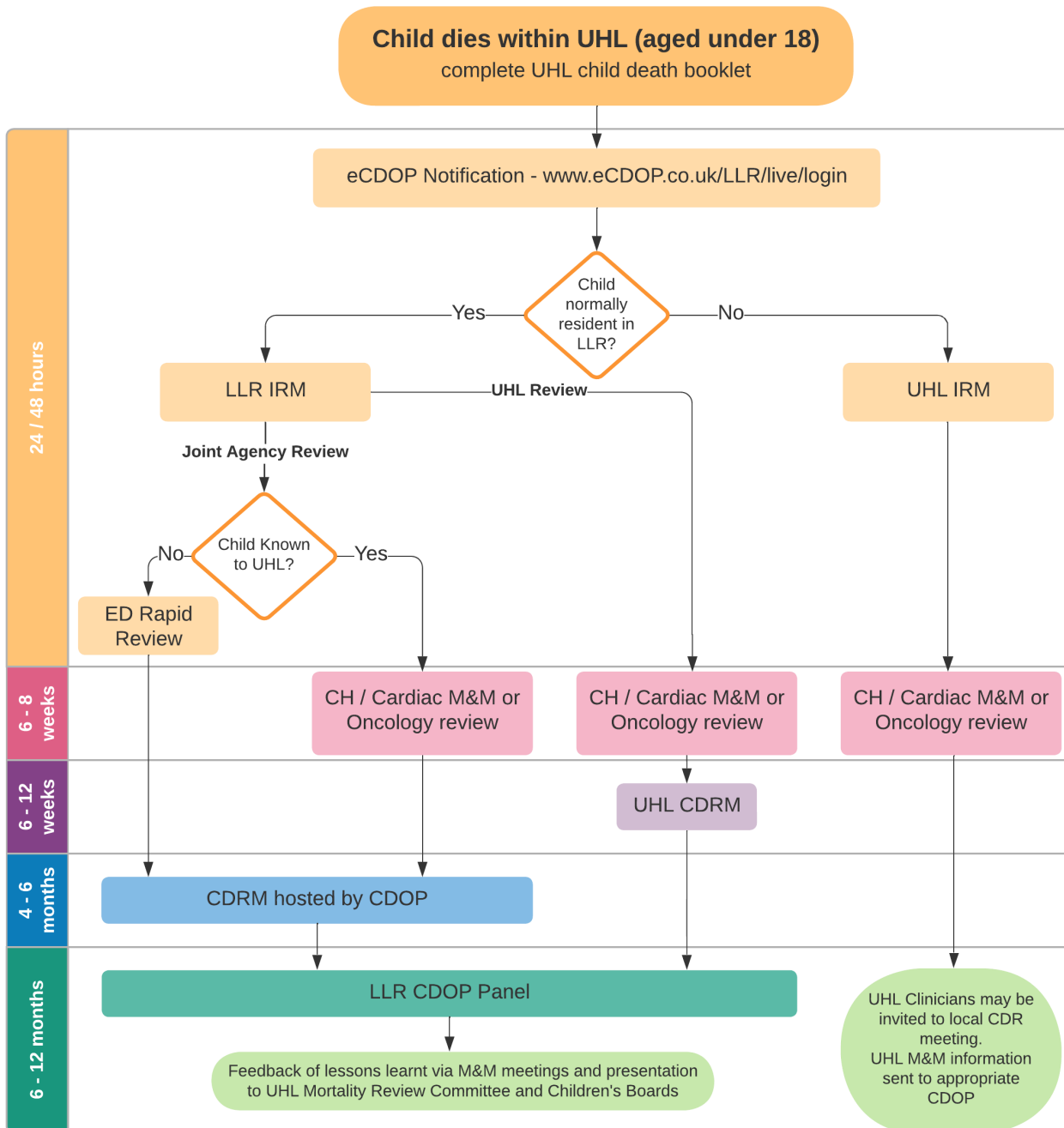


Please ensure that ALL relevant checklists are completed.

You may also find it useful to refer to the UHL guidance on

- Reorientation of Care
- End of life care plans

For any questions or queries (in hours) please contact the UHL Child Bereavement team on ext 17742 who will be able to help.



IRM	- Immediate Response Meeting - Immediate multi-agency (e.g. health, police, CDOP, Social Care) meeting via MS Teams that clinicians should expect to be involved in if a patient of theirs dies. - Purpose: to decide if the case fulfils the criteria for a joint agency response, information sharing, urgent actions, co-ordination of follow up meetings & ensuring family support is in place
M & M	- Mortality & Morbidity Meeting - Review of care within UHL. Will be hosted by the lead area but all who have been involved in the case should be invited to attend - Focuses on key questions: Identify lead clinician / "what went well?" / "any concerns?" / family follow up
UHL CDRM	- Child Death Review Meeting Hosted by UHL - Review of case with all involved in the child's care (inc. GP, health visitor, community team, CAMHS etc.) - Includes any information submitted via eCDOP forms and UHL M&M output - Focuses on identifying contributing factors, learning and actions
CDOP CDRM	- Child Death Review Meeting hosted by LLR CDOP, chaired by the Designated Doctor for Child Death - Review of case with all involved in the child's care. Will include any JAR findings such as the postmortem, serious incident review or any criminal investigations. - All invited UHL clinicians are expected to attend
LLR CDOP	- Leicester, Leicestershire & Rutland Child Death Overview Panel, chaired by Public Health - Multi-agency review of cases focusing on public health interventions and identifying themes - Final actions and learning feedback via UHL M&M meetings and Q&S framework

Contents

The guideline and proforma is divided into the following colour coded sections for ease of use:

****You will not need to complete ALL the sections, but you must complete SECTION 1 to determine which are applicable to the case** Pages that are not applicable should be removed. You will either complete section 1 +2,3,4 OR 1 + 5.**

Please ensure that all original copies are filed in the patient record and that copies are made to accompany the child to the mortuary and for Bereavement Services (as outlined later).

Section	Name	Includes	Pages
One	Resuscitation	Team Members	5
		Pre-Hospital Care	6
		Hospital Care	7
		Confirmation of Death	8-9
Two	History & Family Profile	Family Profile	11
		Family History	12
		Recent Events	13-14
		Past Medical History	15
		Emotional Health & Wellbeing	16-17
		Additional Information	18
Three	Examination and Body Maps	Examination	19
		Body Maps	20-24
		Investigations	25
Four	Checklists (for cases where a MCCD is not issued)	Doctor Checklist	26-28
		Nursing Checklist	29-30
		Final Checklist	31-32
Five	Issuing the Death Certificate (MCCD) and Checklists	Issuing the Certificate	33
		Summary of Care	34-35
		Doctor Checklist	36-38
		Nurse Checklist	39-40
		Final Checklist	41
Appendix	One	Directory of Contacts	43
	Two	Photography and Keepsakes	44
	Three	Hospital Deaths– Cause Unknown	45
		Deaths in Life Limited Children	46
	Four	Joint Agency Response	47
	Five	Removal of Specimens from Deceased	48
	Six	Laboratory Chain of Evidence Form	49
	References		50

Resuscitation-

in ED or Arrest Call on Ward

Children will be brought to the Emergency Department by either parents/carers or ambulance staff. If CPR is being given on arrival continue until there has been sufficient time for an assessment of the situation. If CPR not being given assess:

- colour (dependent discoloration)
- tone (rigor mortis)
- temperature (note it may have been high at time of death; consider immersion hypothermia)
- last known breath

Hospital Number: _____
Surname: _____
First name: _____
Date of birth: _____

If you are unsure initiate effective CPR whilst continuing to make your assessment

When to Stop?

The decision to discontinue resuscitation is a difficult one and should be made by the most senior doctor present. If there is no detectable cardiac output nor any signs of cerebral activity for 20 minutes (including pre-hospital), it is reasonable to stop.

Parental presence during resuscitation-

Parents may wish to remain with the child in the resuscitation room. If this is the case an experienced staff member should be allocated to remain with them explaining procedures throughout. Alternatively they should be taken to a private room nearby and kept informed of events. For out of hospital arrests, one of the team should obtain a brief history at this stage.

Once the child has been pronounced dead the responsibility for breaking the bad news to the parents rests with the most senior member of the team. There should be privacy for the initial period of distress. Parents should be encouraged to hold the child and spend as much time as they wish with them.

Answer questions honestly but do not assume a cause of death as a post mortem may show something unexpected.

Resuscitation Team Members– Name, Grade and Department

(continue over if needed)

[illegible]

Title: Child Death Paperwork & COPD Process (0-18 years) Page 6
V:2 Approved by: UHL Children's Quality & Safety Board: December 2025 Next Review: December 2030
Trust Ref No: D3/2021
NB: Paper copies of this document may not be most recent version. The definitive version is held on InSite in the [Policies and Guidelines Library](#)

Take as much detailed information as you can from the ambulance personnel. This section may be completed by any member of the resuscitation team.

Date of birth: _____

This should include details of the address where the child was collapsed, a description of the child, family and scene on their arrival. Any resuscitation attempts at the scene. Names and contact details of the ambulance personnel who attended.

Ambulance CPR Yes / No commenced ____ :

Resuscitation

Record the important events in the hospital resuscitation, including the timeline.
In ED please also insert Critical Interventions Log

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Primary Survey

Use an ABCDE approach and try to document a core temperature

Endotracheal Tube Placement part A
Should be completed by Dr managing Airway

Tube Size: _____ Length: _____

Attempt no: _____ Grade View: _____

Position Confirmed by (Circle as appropriate):
Laryngoscopy, Chest Movement, Auscultation, EtCO₂

Completed by: _____

Confirmation of Death

Record the date and time of death and how death was confirmed in the boxes at the foot of this page

Hospital Number: _____
Surname: _____
First name: _____
Date of birth: _____

Confirmation of Death

In many cases of sudden unexpected death there are clear signs of death (rigor mortis, hypostasis). In the absence of clear signs of death the child/young person should be observed by the person who is responsible for confirming death for a **minimum of five minutes** to establish that irreversible cardiorespiratory arrest has occurred.

The absence of mechanical cardiac function is normally confirmed using a combination of the following:

- absence of a central pulse on palpation
- absence of heart sounds on auscultation

These criteria will normally suffice however; their use can be supplemented by one or more of the following:

- asystole on a continuous ECG display
- absence of pulsatile flow using direct intra-arterial pressure monitoring
- absence of contractile activity using echocardiography

Any spontaneous return of cardiac or respiratory activity during this period of observation should prompt a further five minutes observation from the next point of cardiorespiratory arrest.

After five minutes of continued cardiorespiratory arrest the absence of the pupillary responses to light, of the corneal reflexes, and of any motor response to supra-orbital pressure should be confirmed.

The time of death is recorded as the time at which these criteria are fulfilled.

In children who are mechanically ventilated the diagnosis of brain stem death should be made in conjunction with the Children's Intensive Care Consultant following the recommendations of local and national guidelines on the diagnosis of brainstem death.

Deaths in Suspicious Circumstances

If you have concerns that a death may be due to unnatural causes i.e. homicide then you must inform the coroner of your concerns **immediately**. You should proceed as instructed by the coroner or their officer. In such cases a Home Office post mortem will be required and a police and section 47 safeguarding investigation will be instigated. In such cases:

- **DO NOT** remove medical devices (such as NGTs, ET tubes or IV/IO Lines)
- **DO NOT** take post-mortem specimens for laboratory testing
- **ANY** samples taken prior to death must be held for discussion with the Police.
- **DISCUSS** with the Police and Senior ED Doctor or Paediatrician present before preparing keepsakes for the family.

Demographic Details

Date of Death / /

Time of Death :

Verified by

Name

Grade & GMC no.

Signature

Death Verification (see above)

- ☐ Absent central pulse on palpation
- ☐ Absent heart sounds on auscultation
- ☐ Asystole on a continuous ECG display
- ☐ Absent pulsatile flow using direct intra-arterial pressure monitoring
- ☐ Absent contractile activity using echocardiography
- ☐ Brainstem death criteria in mechanically ventilated patient

STOP

Not all deaths require referral to the Coroner or the initiation of a Joint Agency Response (JAR). It is important that this is a considered decision and should be discussed with the a Paediatric Consultant (preferably one who knows the child well) and the Medical examiner if you plan to issue a MCCD.

The decision is a two step process:

1. **Is the death is from external causes OR circumstances are unclear OR safeguarding concerns (including children who die in custody or while under mental health section) OR problems with care or service delivery are suspected?**
(This may be the case patients who die in ICU following successful resuscitation in the ED)

YES- The death will need to be referred to the coroner and will need a Joint agency review

Complete Sections 2,3 and 4

NO- Go to question 2

2. **Are you able to issue a Death Certificate (MCCD) for the child?**

YES- Discuss the death with the Medical Examiner to confirm the MCCD can be completed (further guidance on page 33)

Complete Section 5

NO- Discuss the death with the Medical Examiner for further guidance

Complete Sections 2,3 and 4

Family Profile

Record details for key family members including parents and siblings and consider need for medical care.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Parents

Name

Name

DoB

DoB

Address

Address

Tel No:

Tel No:

Child lives with:

Persons with parental responsibility:

Children and Young People in the Household

A surviving twin or young sibling may cause the family concern following the death and urgent arrangements should be made for them to be examined. **In any sudden unexplained infant death, surviving multiples should be admitted**, as per RCPCH/RCPPath guidance for observation for at least 24 hours and investigation for infection, inherited metabolic disease and cardiac conditions undertaken, with follow up planned prior to discharge. In cases where non-accidental injury is suspected, urgent arrangements must be made to examine all vulnerable siblings, especially those < 2 years of age. Arrangements for this examination should be discussed with Social Care in conjunction with the on-call paediatric consultant.

First name

Surname

Age

Lives with

School

Other Key Family Members

Including siblings who may live with other family members and step parents.

Family History

Record the child's family history.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Family History

Please construct a two generation genogram with names, ages and medical conditions Including any medical or psychiatric history of the parents and other immediate family members; infectious contacts; any history of respiratory, cardiac, or neurological disorders in the family and any previous infant or other sudden deaths (child or adult) in the family.

Recent Events

Record the child/young person's recent history of events.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Last Hours (including last sleep and last feed in infants)

If relevant- where and how the patient was put down, clothing, bed coverings, position; any changes in that during the course of the night; if bed sharing, who else was in the bed and their positions relative to the patient; when and by whom the patient was checked during the sleep; description of the last feed, use of pacifier (dummy) and any night time feeds; heating and ventilation.

*Where and how the patient was found, position, coverings, appearance and any unusual features; any action taken after the patient was found. **Consider the use of a diagram to clearly show positions.***

Recent Events (Continued)

Record the child/young person's recent history of events.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Last days and weeks-**Specific Symptoms Review**

	Yes	No	Comments
Coryza			
Diarrhoea			
Vomiting			
Drowsiness			
Cough			
Decreased feeding			
Rash			
Sore throat			
Headache			
Earache			
Crying			
Unsettled			
Fever			

Past Medical History

Record the past medical history as appropriate for the individual patient.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Antenatal, Birth and Neonatal History

Including place of birth, mode of delivery, birth weight, resuscitation required, NICU admissions etc.

Developmental History

Gross Motor

Fine Motor and Vision

Hearing, Speech and Language

Social

Previous Illness and Injuries

Medications

Allergies

Immunisations

Emotional Health & Wellbeing (1)

If felt to be appropriate and/or relevant (e.g. in older children) explore information about the young person's emotional health and wellbeing.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Home

Did they get on with others in the family? Did you argue much? Did they have their own room? How much time did they spend at home? Who did they talk to if they had problems?

Education

Which school did they attend? Were they getting on OK? Any problems?

Activities & Relationships

What did they like to do in their free time? Did they have a job? Do you know their friends? How did he/she like to stay in touch with friends? Did they talk to you about relationships? Do you know if they had a partner? How long had they been together? How old are they? Do they know that he/she has died?

Emotional Health & Wellbeing (2)

If felt to be appropriate and/or relevant (e.g. in older children) explore information about a young person's emotional health and wellbeing.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Drugs / Alcohol

Do you know if any of his/her friends smoked, used drugs or drank alcohol? Do you know if he/she did? Was this something you or they were worried about?

Mental Health

Were they a generally happy or sad person? Did you ever have any worries about how they were feeling? Any problems with eating or sleeping? Do you know if they ever tried to harm themselves?

Safety

Did you have any concerns about their group of friends? Did you feel they were safe when they went out? Did they spend much time on the internet? Any worries about bullying?

Additional Information

Use this space to record any additional information you feel is relevant.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Examination after Death

If possible, complete with the police present, to prevent duplication. A police photographer will also attend.

This examination must be completed by a Paeds consultant OR a ST4+ doctor under the supervision of a Paeds consultant

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

General State of Child

Note presence or absence of rigor mortis, clothing, hygiene, secretions. The child's clothing and nappy should be placed in a property bag and remain with the child.

External Examination

Record any rashes, marks/bruises/abrasions/cuts and pattern of lividity and record on the body map. Include measurements and detailed descriptions whenever possible, utilise the photographer to document relevant marks.

Systemic Examination

Examine for eye, oral and ear abnormalities including Ophthalmoscopy. Perform head, abdominal and limb examination for masses, swelling or deformity.

Ophthalmoscopy findings:

Please tick

Cornea: ☐ Cloudy ☐ Clear

Retina: ☐ Seen ☐ Not Seen

Haemorrhage: ☐ Seen ☐ Not Seen

Blood: ☐ Seen ☐ Not Seen

Additional Details: _____

Record Core Temperature _____ °C at

_____ : _____
Rectal */ Tympanic/ Oesophageal (delete as appropriate)

**DO NOT use rectal temp if any sign of anal trauma.*

Examination — Body Maps

Record any bruises, marks, rashes, bites or other lesions (including cannulation/venepuncture marks) a ruler is available on the following page.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Endotracheal Tube Placement - part B

To be completed by Doctor performing examination

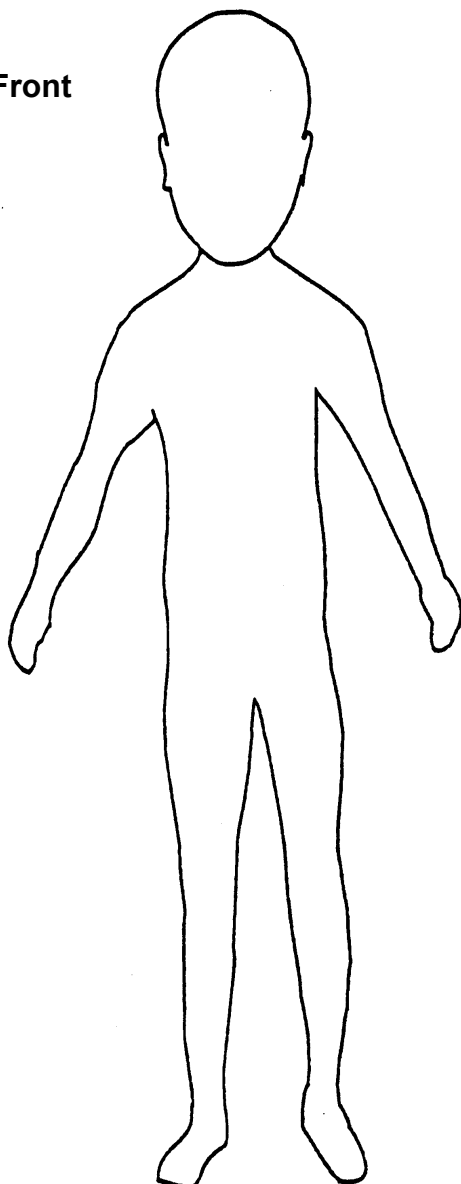
On confirmation of correct tube placement by a 2nd Dr (trained in direct laryngoscopy) endotracheal tubes **CAN BE REMOVED** to reduce distress to the family.

If unsure, **DO NOT** remove endotracheal tube (ET).

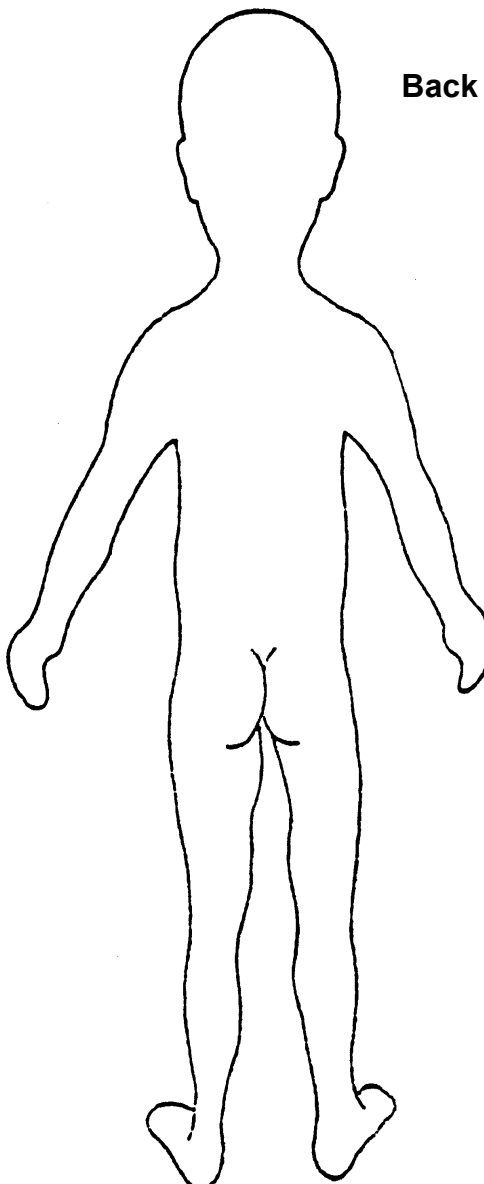
ET visualised passing through vocal cords: Yes ☐ No ☐ sign: _____

Further Details (If No selected): _____

Front



Back



Examination — Body Maps

Record any bruises, marks, rashes, bites or other lesions (including cannulation/venepuncture marks)

Completed by: _____

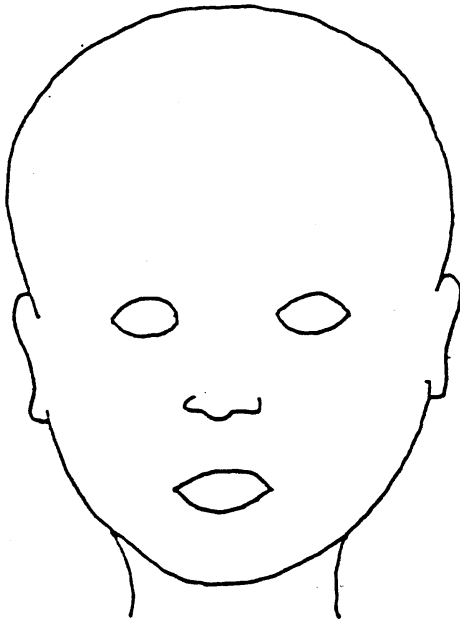
Hospital Number: _____

Surname: _____

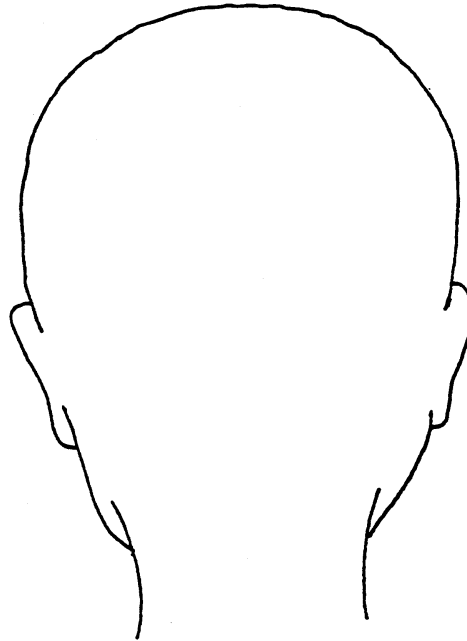
First name: _____

Date of birth: _____

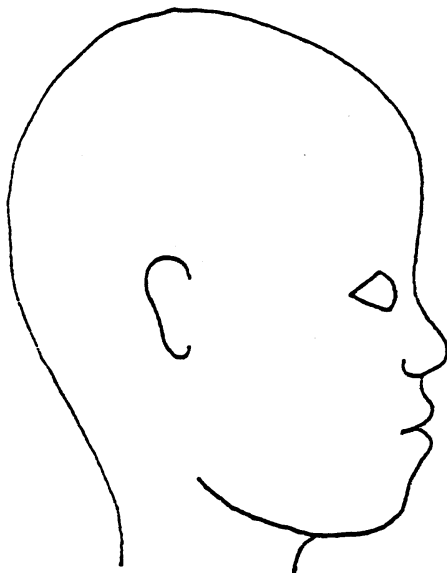
Please use the ruler below to assist in measurements



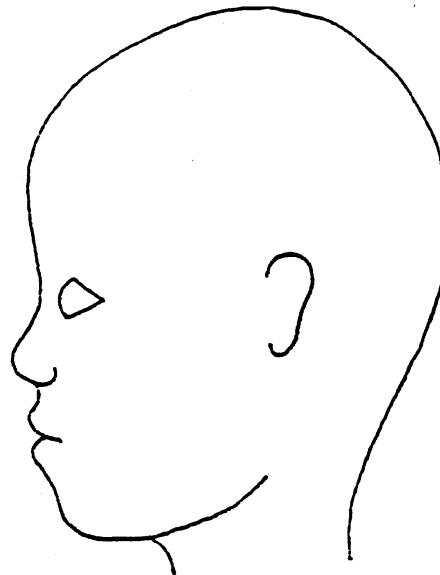
FRONT



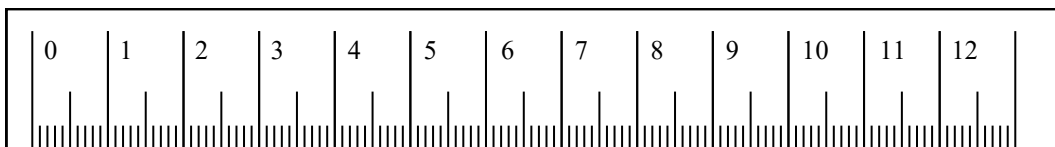
BACK



RIGHT



LEFT



Examination — Body Maps

Record any bruises, marks, rashes, bites or other lesions (including cannulation/venepuncture marks)

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

The form contains four hand diagrams arranged in a 2x2 grid. The top row shows the back of the hands, with the right hand on the left and the left hand on the right. The bottom row shows the palms of the hands, with the right hand on the left and the left hand on the right. Each diagram is a simple line drawing of a hand with fingers spread. The labels 'R' and 'L' are placed below each hand, and the labels 'BACK' and 'PALM' are placed between the columns.

Examination — Body Maps

Record any bruises, marks, rashes, bites or other lesions

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

RIGHT

LEFT

Female

RIGHT

LEFT

Male

Examination — Body Maps

Record any bruises, marks, rashes, bites or other lesions (including cannulation/venepuncture marks)

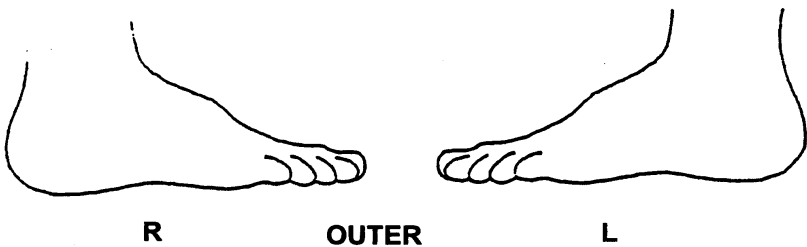
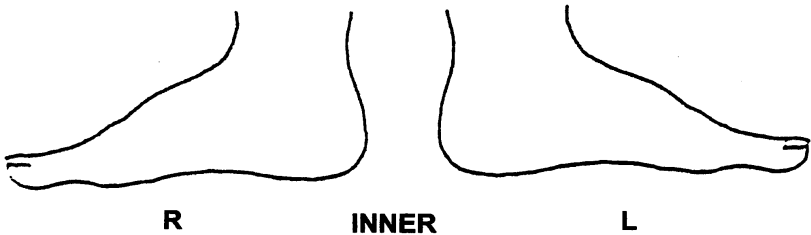
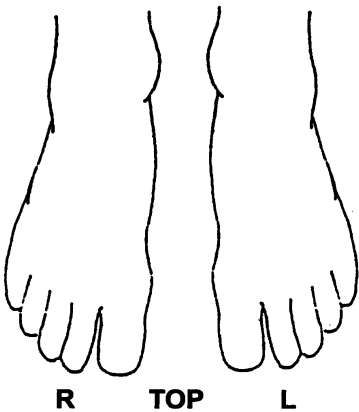
Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____



Summary of Medical Devices in Situ

Device	Position

Post Mortem Investigations

Record the investigations performed both during resuscitation and after certification of death.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Laboratory Investigations Following Certification of Death

In Line with the Human Tissue Act, the coroner has agreed that time critical post mortem samples **SHOULD be taken in the EMERGENCY DEPARTMENT**. All other samples will be gathered at the time of the post mortem by the pathologist who is acting on behalf of the Coroner. The PM may occur up to 2 weeks after death. **If any other investigations eg toxicology, metabolic tests are considered urgent please discuss with the coroners office.**

If limited bloods available, please prioritise Blood Culture and Gas.

Samples Taken

Blood tests	Culture	☐	Urea and Electrolytes	☐
	Blood Gas	☐	C-Reactive Protein	☐
	Full Blood count	☐	Mast Cell Tryptase (in cases of possible anaphylaxis/ asthma)	☐
L.P.	Microscopy & Culture	☐		
	Protein & Glucose	☐		
	Virology	☐		
Pernasal Swab	Pertussis PCR	☐		
Throat/ Nose	Microscopy & Culture	☐	Extended Respiratory Panel	☐
Urine Sample	Microscopy & Culture	☐		

The assumption is made that ALL ED deaths will have postmortem samples taken. Please document below reasons if not completed-

☐ Unable to obtain

☐ Not clinically indicated (i.e. suicide/ RTC) - provide explanation-

Name-

*In suspicious circumstances, DO NOT take post mortem samples. Any samples obtained prior to death should be discussed with the Police.

Remember to document the site of any puncture marks made after death on the body map, for example, please mark the site at which a femoral puncture or lumbar puncture performed.

If you have any questions please contact the doctor in charge of CED.

Doctor Checklist 1

Record whether the following have been completed and by whom. Everything must be done unless you document a reason why it has been omitted.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Parents informed of death and appropriately supported	<i>Ensure an appropriate setting is available</i>			
Inform the on call Paediatric Consultant and ED consultant	<i>If not already present/ aware.</i>			
Consider the need for immediate team debrief	<i>If possible debrief to be led by an independent Consultant. Remember to include paramedic and other allied health professionals. Signpost staff to Trauma Risk Management Service for on going support</i>			
Inform the Police of the child's death to initiate the Joint Agency Response (24/7)	Call 0116 222 2222 and ask the automatic switchboard for Child Abuse Investigation Unit . You should speak to the on-call DI for Child Deaths . If unavailable, leave a number for them to call back on. Two officers will attend UHL			
Inform the Coroner of deaths where death is from external causes, the circumstances are unclear, or safeguarding concerns or problems with care or service delivery are suspected.	<i>Discuss with the Police on arrival WHO will contact the coroner.</i> <i>The police will contact for the majority of cases.</i> If the police inform then no further notification is required from UHL			
Ensure PM sample page completed Would this child benefit from urgent metabolic or genetic testing?	<i>Please comment if a PM skin biopsy for fibroblast culture (done in mortuary) would be useful in this child. This will be discussed further in IRM</i>			

Doctor Checklist 2

Record whether the following have been completed and by whom. Everything must be done unless you document a reason why it has been omitted.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Multiagency STRAT Call to consider Safeguarding	<i>Police, Senior Doctor and City/County Safeguarding should have a joint call regarding any urgent safeguarding needs. In hours UHL safeguarding team should be invited to attend.</i>			
Is this a Breastfeeding mother?	<i>Please provide a copy of the 'Lactation After Loss' leaflet. If baby <28 days – contact bereavement midwives on uhl-tr.wnbereavementlink@nhs.net for additional support</i>			
Check Nervecentre for previous attendances				
Parents /Carers seen and detailed History taken	<i>Please see Section 2 for assistance in completion and recording of detailed history Should be done jointly with Police and Senior Dr</i>			
Perform detailed examination of Child	<i>Please see Section 3 for completion of detailed examination and recording Should be done jointly with Police and Senior Paediatrician</i>			
Complete an eCDOP notification	<i>https://www.ecdop.co.uk/LLR/live/login Click on the GREEN button PRINT a copy of referral for notes</i>			

Doctor Checklist 3

Record whether the following have been completed and by whom. Everything must be done unless a documented reason why it has been omitted.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Notify the Patients' GP	<i>In Hours: use GP details on ED Notes</i> <i>If out of hours: for LLR children the CDOP team will pick up next working day</i>			
Document and notify other involved agencies ie DIANA team	<i>To ensure the family don't have visits from teams unaware of the death.</i> <i>List all agencies involved.</i> <i>Appendix 1 for contact details</i>			
Complete MBBRACE form (For infants less than 28days) Fill in as much info as able (sections 1.1, 1.2, 5.1 and 6.1 are the essential info)	<i>Contact Maternity Bleep Holder via switch board to inform of death</i> <i>www.npeu.ox.ac.uk/assets/downloads/mbrace-uk/2019%20Perinatal%20data%20collection%20form%20-%20MAIN%20v4.1.3.pdf</i> <i>Scan form & email to uhl-tr.perinatalreviewmailbox@nhs.net</i>			
Ensure notes and Child Death paperwork completed	<i>A copy of this booklet should be placed in the notes & one sent with the child to the mortuary in a sealed envelope.</i> <i>In ED scan onto CITO</i> <i>Copy offered to POLICE</i>			
Complete and submit Datix incident report for ALL 2222 calls	<i>Available via UHL Connect</i>			

Nursing Checklist 1

Record whether the following have been completed.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Interpreter contacted for family if required	Do not use a family member. Use D A Languages via 0330 088 2443 Your department PIN is required. Police may provide face to face interpreter. See UHL Connect for further info			
Other family members informed	At request of family present only			
Parents/Carers given opportunity to hold their child	Nurse to maintain discreet, supportive presence at all times			
Child laid out Appropriately Name bands placed on child (x2)	Do not remove lines/ medical devices in suspicious circumstances, if in doubt liaise with Senior Attending Doctor and refer to last offices policy.			
Family offered lock of hair, hand and footprints (available within the memory box)	Record where lock of hair taken from Clear liquid printing kit can be used even in suspicious circumstances.			
Appropriate religious or cultural needs met and religious representation contacted at family's request	On-call Multi-faith chaplaincy support available LRI 15487 / 16754 or via switchboard			

Nursing Checklist 2

Record whether the following have been completed.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Family given Memory box	Give paperwork and box separately UHL Child Bereavement Booklet *complete details page* Butterfly condolence card			
Management of property and valuables	Property/Valuables:	Delete as appropriate: i) Given to Family ii) Bereavement Services iii) Police Record details of receiving person.		
Mortuary and Porters Contacted	Mortuary: LRI 15596 Porters: Contact via Navenio portal or helpdesk 17888 Ensure appropriate sized trolley requested for patient transport			
Reception/ ward clerk notified to cancel all appointments				
Email notification sent to UHL Child Bereavement Team	Please include next of kin details (name & address)- Uhl- tr.childbereavement@nhs.net This will ensure a letter is sent at 6 weeks offering bereavement follow up.			
Suitable transport arranged for family	For ED deaths -QR code tickets available to exit carparks for free from Tracker			
Complete UHL notification of Death form Yellow bereavement care check list	Copy in notes Copy with child			

Final Checklist

Ensure the following have been completed

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Doctor Checklist completed				
Nursing Checklist completed				
Notes and UHL Child Death Paperwork completed and copied	Originals to remain with the patient notes and scan onto CITO <i>Copies to:</i> 1) Accompany the patient to the mortuary (sealed envelope) 2) ED deaths- place copy in RIP drawer 3) ALL deaths- copy scanned and emailed to Uhl-tr.childbereavement@nhs.net 4) Copy offered to Police (acting on behalf of coroner)			
UHL Notification of Death Form Completed (Yellow bereavement care checklist)				
Patient discharged from Nervecentre				

Background

Paediatric deaths are thankfully rare and as such the processes that staff on the wards will need to go through are infrequent. The guidance in this section is aimed to help staff navigate the process and ensure the Medical Certificate of Confirmation of Death (MCCD) and any other paperwork is completed correctly. It is best practice that the certificate should be completed by the Child's named consultant. If a number of consultants cared for the child there may need to be a discussion around the cause of death and any contributory factors.

Medical Examiner

ALL cases must be discussed with the Medical Examiner prior to issuing a MCCD. This is to ensure that there is no need to refer to the coroner. In addition, when there is a planned withdrawal of active care, or an expectation that the child may die over a weekend, it is possible to discuss the case with the ME prior to death.

The full guidance on the roles and responsibilities regarding issuing of MCCD and also cremation certificates and early release of the body can be found in the [Medical Examiners UHL Policy](#).

While the majority of cases that require a Joint Agency Response (ie police involvement) and referral to the coroner will be deaths where the cause is unknown, this is not always the case. Discussion of all cases with the ME will ensure that we do not miss cases that need to be referred. Take for example a child who dies of a hypoxic brain injury following a drowning. This child will need to be referred to the coroner and also have police involvement due to the circumstances, even if the cause of death is known. There is further guidance and examples in [Sudden unexpected death in infancy and childhood Multi-agency guidelines for care and investigation](#).

ALL child deaths must have a eCDOP referral completed (details in the following checklists).

MCCD

The MCCD wording should be documented below:

1a

1b

1c

2

Examples of cause of death section from MCCDs:

1 (a) Disease or condition leading directly to death

Intraperitoneal haemorrhage

1 (b) other disease or condition, if any, leading to 1(a)

Ruptured metastatic deposit in liver

1 (c) other disease or condition, if any, leading to 1(b)

primary adenocarcinoma of ascending colon

2 Other significant conditions contributing to death but not related to the disease or condition causing it

Non– insulin dependent diabetes mellitus

Summary of Care

If the child died following an arrest on the ward please ensure you have completed SECTION ONE to document the resuscitation attempts.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Named Consultant-

Other Consultants involved in Care-

Cause of Death-

Past Medical History-

Brief Summary of Events Leading Up to Death-

Summary of Care (Continued)

Record the child/young person's recent history of events.

Completed by: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Family Structure-

Please construct a two generation genogram with names, ages and medical conditions Including any relevant medical history in the parents or other immediate family members and any previous infant or other sudden deaths (child or adult) in the family.

Religion-

Did the child have a Children's and Young Person's Advanced Care Plan and/ or ReSPECT form in place? Please circle

YES

NO

Are the family likely to request a quick release of the body for religious/ cultural reasons? Please circle

YES

NO

Are you certain that the death is NOT from external causes, that the circumstances are clear, there are NO safeguarding concerns, NO problems with care and NO issues with service delivery? Please circle

YES

NO**

**** If you have answered NO then you should not issue a MCCD until the case has been discussed with both the ME and the Coroner.**

Doctor Checklist 1

Record whether the following have been completed and by whom. Everything must be done unless there is a documented reason why it has been omitted.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Parents informed of death and appropriately supported	<i>Ensure an appropriate setting is available</i>			
Inform the on call Consultant	<i>If not already present/ aware.</i>			
Consider the need for immediate team debrief	<i>If possible debrief to be led by an independent Consultant.</i>			
Are the family likely to request a rapid release? (ie for religious reasons or transfer to Rainbows)	<i>In hours contact the mortuary and bereavement services to alert them.</i> <i>Out of hours please inform the duty manager so they are aware.</i>			
Would the family like to take their child to Rainbows bereavement suite?	<i>Contact Rainbows to ensure that this can be facilitated before offering to the family.</i> <i>01509 638000</i> <i>Written guidance is available on trust website</i>			
Discussion with Medical Examiner	<i>In Hours call 07815 028098</i> <i>Out of hours please identify who will be responsible for discussing the death the next day</i>			
Issuing of the Death Certificate +/- Cremation form part 1	<i>Following discussion with the relevant Consultants involved in the child's care, record who has issued the MCCD and liaise with Bereavement services about cremation form</i>			

Doctor Checklist 2

Record whether the following have been completed and by whom. Everything must be done unless there is a documented reason why it has been omitted.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Complete MBBRACE form (For infants less than 28days) Fill in as much info as able (sections 1.1, 1.2, 5.1 and 6.1 are the essential info)	<i>Inform Maternity Bleep Holder via switch board to inform of death</i> www.npeu.ox.ac.uk/assets/downloads/mbrace-uk/2019%20Perinatal%20data%20collection%20form%20-%20MAIN%20v4.1.3.pdf <i>Scan form & email to uhl-tr.perinatalreviewmailbox@nhs.net</i>			
Complete an eCDOP notification	https://www.ecdop.co.uk/LLR/live/login Click on the GREEN button PRINT copy of referral for notes			
Is this a Breastfeeding mother?	<i>Please provide a copy of the 'Lactation After Loss' leaflet.</i> <i>If baby <28 days – contact bereavement midwives on uhl-tr.wnbereavementlink@nhs.net for additional support</i>			
Ward Clerk to cancel all future OPA	<i>Consider external agencies who may visit and need to be informed (ie physio, Diana)</i>			
Hospital Postmortem	<i>Best practice is that all families should be offered a hospital PM</i> <i>Please see guidance on trust website on process</i>			
Tissue Donation	<i>In children where death was expected, organ and tissue donation may have been discussed already. Please document if it has, and consider discussing tissue donation if not completed already.</i>			

Nursing Checklist 1

Record whether the following have been completed.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Interpreter contacted for family if required	Do not use a family member. Use D A Languages via 0330 088 2443 Your department PIN is required. See UHL Connect for further info			
Other family members informed	At request of family present only			
Parents/Carers given opportunity to hold their child	Nurse to maintain discreet, supportive presence			
Family offered lock of hair, hand and footprints (available within the memory box)	Record where lock of hair taken from Clear liquid printing kit can be used even in suspicious circumstances.			
Appropriate religious or cultural needs met and religious representation contacted at family's request	On-call Multi-faith chaplaincy support available LRI 15487 / 16754 or via switchboard			
Child laid out appropriately Name bands placed on child (x2)				

Nursing Checklist 2

Record whether the following have been completed.

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Family given Memory box	<i>Family may wish to collect later from Bereavement services</i>			
Family given UHL Child Bereavement booklet	<i>Ensure details page is completed and give any additional info needed (ie post mortem information leaflet)</i>			
Write and post Butterfly Condolence Card to next of kin	<i>This should be signed from the ward</i>			
Ask family to telephone Bereavement Services office next working week day (Mon– Friday) regarding certification	<i>Numbers inside 'Following the Death of your Child' booklet</i>			
Notes to Bereavement Services Office	<i>Please take ASAP to allow the ME time to review notes before the MCCD is issued</i>			
Notification to UHL Child Bereavement Team	<i>In hours– ext 17742 / 16776</i> <i>Out of hours- Uhl-tr.childbereavement@nhs.net</i> <i>Please give details of NofK</i> <i>The team can offer support to ward staff and will send follow up letters at 6/52.</i>			

Final Checklist

Ensure the following have been completed

Completed by: _____ Role: _____

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

Action	Details	Comments	Time	Sign when completed
Doctor Checklist completed				
Nursing Checklist completed				
UHL Child Death Paperwork completed and copied	<i>Originals to remain in the patient notes</i> <i>Copy to:</i> <i>1) Accompany the patient to the mortuary (sealed envelope)</i> <i>2) Ward clerk to scan and email to Uhl-tr.childbereavement@nhs.net</i>			
Mortuary and Porters Contacted	Mortuary: LRI 15596 Porters: Contact via Navenio portal or helpdesk 17888 <i>Ensure appropriate sized trolley requested for patient transport</i>			
UHL Notification of Death Form Completed (Yellow bereavement care checklist)				
Patient discharged from Nervecentre				

Directory of Contacts

University Hospitals of Leicester 0300 303 1573		
Name	Number	Email/ additional info
UHL Child Bereavement Team	Ext 17742 / 16776	Uhl-tr.childbereavement@nhs.net
CDOP office	0116 295 8724 or 07876034566	Please leave message out of hours
UHL Safeguarding Team	0116 258 5770	
Social Care	City: 0116 454 1004 County: 0116 305 0005	
Mortuary	LRI 15596	Out of hours: contact duty manager via switch
Porters	Contact via Navenio Portal or Helpdesk 17888	
Medical Examiners Office	07815 028098	uhl-tr.medicalexaminersmailbox@nhs.net
Coroner's Office	Via Switchboard	
Bereavement Services	Ext 15194 / 15196	
Leicestershire Police	0116 222 2222	
D A Language Services	0330 088 2443	You will need dept Pin to access Please see UHL Connect for more info
East Midlands Ambulance Service	0115 884 5000	
Rainbows Hospice	01509 638000	
Diana Service	0116 295 5080	(answer phone out of hours)
Muslim Burial Council Leicester	07803 240493	info@mbcol.org.uk
Trauma Risk Management (TRiM)		uhl-tr.trimsupportmailbox@nhs.net Support for staff following traumatic events

Photographing and collecting footprints and hair locks from a deceased child

Taking photographs of a deceased child or young person

- **Parents/Carers can take photographs with their own camera or phone for bereavement purposes.**
- Under no circumstances should photographs be taken using departmental or staff cameras or phones.
- Photographs are to be taken for bereavement purposes only. NEVER for forensic, personal or educational reasons.
- The Police will now send a photographer to ALL unexpected deaths to aid documentation.

Protocol for collecting hand/footprints and hair locks from a deceased child

- **In suspicious circumstances, discuss with senior attending doctor before proceeding.**
- Obtain consent from parents and document in patient notes.
- Collect the imprint wipes, sensitised certificates and sample bag from the bereavement box.
- Open the foil-sealed wipe, and wipe the infant's hand/foot. NB the wipe feels dry, this is normal.
- Gently, but firmly press the infant's hand/foot onto the sensitised certificate.
- The wipe can be used for several prints.
- Cut a small sample of hair from the child's head and place in sample bag.
- Hand over the prints and/or hair lock to the family. If they would prefer, put them in an envelope with accompanying patient details to be given at a later date in the SUDIC process.
- Equipment for hair lock and inkless hand and foot prints are available within the memory boxes. Please note that Clay moulds/ other ink prints should only be used before a child dies.

Hospital Deaths— Cause Unknown

The process of responding to deaths in the Emergency Department is well established and usually a clear cut process. It is important to remember however that sudden deaths can occur in hospital where a cause is unknown. Equally a child may die a number of days to weeks following an event which would have triggered a joint agency response, had they died in the Emergency department. The response to these deaths should be as rigorous as any death in the community but may be modified depending on the circumstances.

The CDOP team will work closely with the responsible consultant to manage the response to the child's death.

Initial Actions

The consultant on-call for acute paediatrics should attend to help coordinate the response and support the family and staff. The coroner should be informed as soon as possible within working hours or immediately if there are concerns that the death may be suspicious. Nursing and medical staff who were on shift when the infant/child died should document what happened before the end of their shift with reference to the preceding pages and the relevant sections of this proforma and completing the relevant checklists. If the police are already aware of the case they should be immediately made aware that the child has died.

Initial Response

An eCDOP referral should be completed as soon as possible to allow coordination of an initial review meeting with all the relevant parties. The police should be contacted and informed of all unexpected deaths immediately (in their role as coroner's officers) and a discussion should be had as to whether they need to visit the ward. Where there are no specific concerns this may not be necessary, but the discussion must still take place. If the police feel a joint home visit (by the police and a CDOP practitioner) is necessary they will liaise with the CDOP team. Social care should also be informed about the death.

Preservation of the Scene

Concerns may be raised, for example where there is suspicion that equipment may have been tampered with, unusual behaviour by family members or staff or serious child protection concerns have already been identified. **Until this has been clarified the cot/bed where the child died should be left undisturbed.** If this is not possible (e.g. the child was on an open ward) the cot/bed clothes should be kept separately until police give further instructions. Fluids, syringes, drugs and other nursing or feeding equipment used should not be discarded until the decision about the police visit has been clarified.

Child Death Initial Meeting

A multi-agency discussion /meeting will be held at 12:30 via MS Teams at the next available slot (always within 48 hours of death). The CDOP team will coordinate this discussion/meeting. If the family are known to social care consideration will be given as to whether a strategy meeting is required.

There should be explicit discussion about the possibility of neglect or deliberate injury contributing to the child's death either by parents or staff.

The paediatrician responsible for the child's care should also form part of the discussion/meeting to provide factual information of the events leading to the collapse/death. The Trust will need to consider independently of the CDOP process whether the case needs further internal investigation via the SUI (serious untoward incident) process. This is a separate process to the Child Death Review and one may inform the other. Multi agency meetings should be minuted.

Hospital Deaths– Cause Unknown (Continued)

There should also be a discussion as to who would be the most appropriate person to support the family after the death and share information with them regarding the post mortem and further investigations. This is usually the lead consultant if the child had one or the paediatrician that dealt with the death.

Final Review Meeting

The death will follow the normal CDOP process. Information from the hospitals own M&M and CDR meetings will feed into this final review. Bereavement support should be offered by the paediatrician involved with the death, this will allow for any questions from the family.

Sudden and Unexpected Deaths in Children & Young People with Life Limiting Conditions

Children with recognised life limiting conditions are usually cared for by a multi-agency team led by a Paediatrician. Some will have a documented Children's and Young Person's Advanced Care Plan or ReSPECT form shared with all agencies involved. The death of these children may not be anticipated that day, but thought to be a significant possibility at some point. However, many children who are at risk of rapid deterioration or sudden death do not have a formal Children's and Young Person's Advanced Care Plan in place even though their death may be anticipated. The response to all these deaths needs to be as rigorous as any other deaths and the normal process followed.

The attending Paediatrician will work closely with the responsible consultant to manage the response to the child's death and help determine whether a cause of death is known.

If the child has died unexpectedly they will be brought to the Emergency Department (unless there is a documented Children's and Young Person's Advanced Care Plan stating otherwise). The consultant on-call for paediatrics should attend to help coordinate the response. The paediatrician usually caring for the child should be contacted as soon as possible (usually in working hours). The coroner should be informed as soon as possible within working hours or immediately if there are concerns that the death may be suspicious.

The CDOP team will work closely with the Paediatrician responsible for the child who will have detailed knowledge of the medical and social issues for the child. The eCDOP notification will trigger the initial response meeting.

The police should be informed and discussion taken place (in their role as coroner's officers) as to whether they need to visit the place where the child collapsed/ died. Children with complex problems are more likely to be cared for in a number of environments and death may not necessarily occur at home. Where there are no specific concerns this may not be necessary, but discussion must still take place. If the police feel a joint visit is necessary, consideration needs to be given as to whether carers and or parents are present for this. Social care should be informed about the death.

Following these discussions it may be possible to issue a death certificate and de-escalate any need for a joint agency response. Where there is some concern about the exact nature of the death or underlying concerns have been identified, the death will trigger a joint agency response.

Joint Agency Response

Guidance taken from Child Death Review Statutory and Operational Guidance-

A Joint Agency Response should be triggered if a child's death:

- is or could be due to external causes
- is sudden and there is no immediately apparent cause (incl. SUDI/C)
- occurs in custody, or where the child was detained under the Mental Health Act
- where the initial circumstances raise any suspicions that the death may not have been natural
- in the case of a stillbirth where no healthcare professional was in attendance.

In any of these circumstances, the on-call health professional, police investigator, and duty social worker should be contacted immediately so as to initiate the joint agency response.

A Joint Agency Response should also be triggered if such children are brought to hospital near death, are successfully resuscitated, but are expected to die in the following days.

In such circumstances the Joint Agency Response should be considered at the point of presentation and not at the moment of death, since this enables an accurate history of events to be taken and, if necessary, a 'scene of collapse' visit to occur.

Removal of Specimens from Deceased Children

The removal of pathology specimens from deceased infants and children in the Accident and Emergency department of hospitals for microbiological, biochemical and toxicological testing is as described in the joint RCPCH and RCPaTH guidance- SUDIC Multiagency Guidelines for Care and Investigation.

Blood samples should be taken using a femoral or brachial puncture, cardiac puncture should be avoided. Ensure it is clearly documented on the body map where any sample has been taken from.

The making of a post-mortem examination and the removal of material from a deceased person for certain purposes including determining the cause of death are licensable activities under the Human Tissue Act 2004. Therefore, in compliance with the HT Act, removal of pathology specimens from deceased infants and children for microbiological, biochemical and toxicological testing must take place on licensed premises.

It is important to note that the Designated Individual (DI) named on the HTA licence has statutory responsibility for ensuring that the premises are 'suitable' and that 'suitable practices' take place on those premises, as well as for complying with the conditions of the licence.

Consent

The HT Act 2004 is clear that removal of tissue from the deceased for the purpose of determining the cause of death can only take place under the authority of the coroner (or, in the case of hospital post mortems only, with appropriate consent). **UHL have been given permission from the Leicester coroner to take the samples stated on page 25 of this guidance for deaths that occur in the emergency department.** However, it is important to stress that removal of other specimens without such coronial authorisation would be unlawful. As mentioned earlier, where it is not a coroner's post mortem case, appropriate consent (from a nominated representative if there is one or next of kin) would be required for the removal of tissue to take place.

From information we have received, it appears there may be instances when samples are removed from a deceased child prior to the death being reported to the coroner. Where this is indicated, the guidance above should be re-iterated and it should be stressed that it is unlawful to remove material without the necessary consent or coronial authority or to remove material in these circumstances on unlicensed premises. These are offences under the HT Act, which can result in a fine, imprisonment or both.

Laboratory Chain of Evidence Form

For use in Deaths in Suspicious Circumstances

Please complete a separate form for each specimen (Photocopy as necessary)
Staple the chain of evidence form to request form and sealed sample bag

Hospital Number: _____

Surname: _____

First name: _____

Date of birth: _____

- **In suspicious circumstances, there must be a clear chain of evidence from the person taking a sample to it's arrival in the lab and processing.**
- For each sample, a **separate chain of evidence form** must be completed and stapled to the request form/sealed specimen bag
- Different laboratory samples (Haematology, Biochemistry, Microbiology etc) are sealed in separate bags.
- The chain of evidence forms are to stay with the samples **at all times**, and to be signed by each person who handles them.
- All samples are to be delivered to the laboratory **via porters only**, and not via the air tube system.

ALL NAMES MUST BE ACCOMPANIED BY A SIGNATURE

Procedure	Name	Signature	Date	Time
Specimen taken by:				
Specimen delivered to laboratory by:				
Received by lab personnel:				
Senior lab personnel check at receipt:				
Senior lab personnel on completion of processing:				

References

Document developed and based on the Child death guidance of Nottingham University NHS Trust, with many thanks.

- RCPCH and RCPaH guidance- SUDIC Multiagency Guidelines for Care and Investigation.

<https://www.rcpch.ac.uk/resources/sudden-unexpected-death-infancy-childhood-multi-agency-guidelines-care-investigation>



- Child Death Review– Statutory and Operational Guidance

https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/859302/child-death-review-statutory-and-operational-guidance-england.pdf

